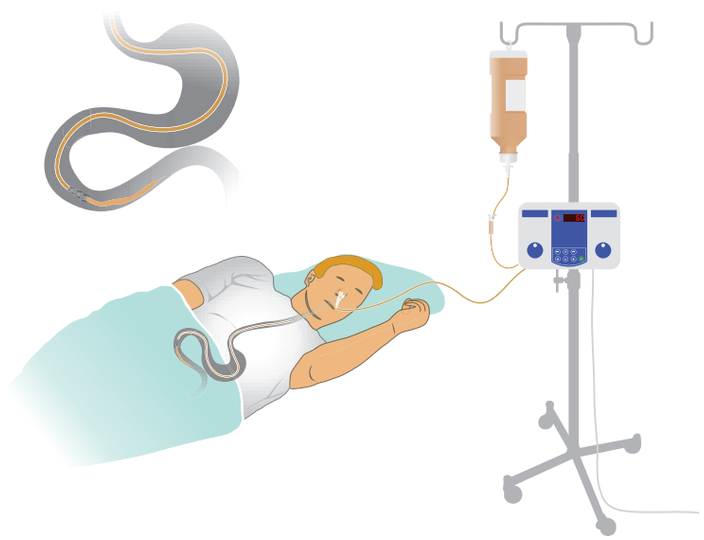
Procedure:

1. Wash hands
2. Explain the procedure to the patient, if patient is conscious
3. Provide privacy to the patient
4. Position the patient in cardiac position
5. Place the mackintosh and towel across the chest
6. Measure the length of the Ryle's tube from tip of the nose to ear lobe to xiphoid process (sternum) and mark with tape
7. Lubricate the Ryle's tube and insert through any nostril posteriorly to the throat
8. Encourage the patient to swallow by giving small sips of water if patient is conscious. For unconscious patient, nurse has to insert tube at the back of throat using tongue depressor
9. Keep the tip of the tube inside the bowl of water and watch for any air bubbles
10. Attach 10/20ml syringe to end of the tube and aspirate back on the syringe gently to obtain gastric juice
11. Push 5-10 ml of air while auscultating the stomach
12. Clean the Ryle's tube after confirming placement
13. Fix the tube with adhesive tape
14. Administer tube feeding, pinch the proximal end of the tube. Attach syringe to the end of the tube and elevate 45cms above head. Fill the syringe with liquid diet, allow to empty gradually
15. And pour water to wash off feed particles

After care of the patient and articles:

- ❖ Remove the mackintosh and towel and disinfect it with antiseptic solution
- ❖ Give comfortable position to the patient
- ❖ Replace the articles
- ❖ Wash hands
- ❖ Record the procedure with date and time
- ❖ Inform any reaction to the incharge sister

The feeding tube controlled by electronic pump passes through the nose, throat and oesophagus, continues through the stomach, and ends in the first section of the small intestine.



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